

Comprehensive Pharmacy Services for Oncology Patients from an Integrated Health-System Specialty Pharmacy

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BACKGROUND

- Vivo Health Pharmacy, an integrated health-system owned specialty pharmacy, holds URAC/ ACHC accreditation, with ACHC Oncology Distinction since 2019.
- The oncology program provides wraparound pharmacy support services, starting with ambulatory pharmacists embedded in the oncology practice.
- Specialty pharmacists at the integrated health-system specialty pharmacy (HSSP) review prescriptions for appropriateness and screen for drug interactions.
- The Oncology Concierge (OC) team supports Prior Authorization (PA) and Financial Assistance (FA) when applicable.
- The specialty Patient Management Program (PMP) provides assessments, counseling, education, and safety monitoring upon treatment initiation and every 6 months, in addition to monthly adherence calls.

OBJECTIVES

This study assesses the effectiveness of the Concierge team in PA approval and FA, and the rate of discontinuation due to adverse drug reactions (ADR) and hospitalizations in patients enrolled in the specialty PMP.

METHODS

- Data were obtained from the electronic health record, concierge program data, and pharmacist assessments from 2023-2024.
- PA and FA data included all patients serviced, regardless of dispensing pharmacy or PMP enrollment.
- Patients were included in the analysis if ≥ 18 years of age, had an oncology diagnosis, and at least 3 fills of an oral oncolytic at the integrated HSSP.
- PMP results included patients opted into the program, and a paired analysis was completed with patient reported outcomes from the semiannual assessments.

RESULTS

Fig 1. Prior Authorization Outcomes (N=12,233)

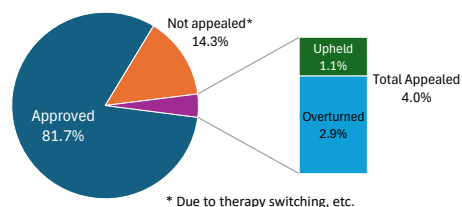


Fig 3. Financial Assistance Savings

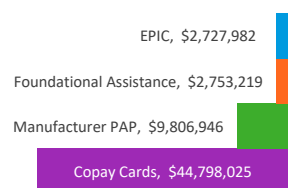


Table 1. Baseline Characteristics of PMP (N=1,071)

Female sex	49.7%
Average age	68.0 years (SD 14.2)
18-64	27.9%
65-79	51.8%
>80	20.4%
White	53.3%
Black or African American	14.0%
Asian	13.0%
Other	13.0%
Unknown/Declined	6.7%

Fig 2. PA Turnaround Time for Approvals (N=9,997)

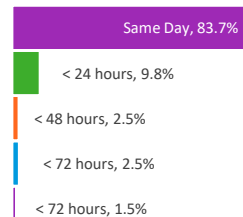


Fig 4. Discontinuation Rate Due to ADR (N=2,364)

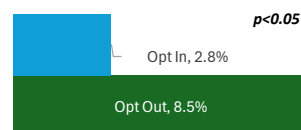
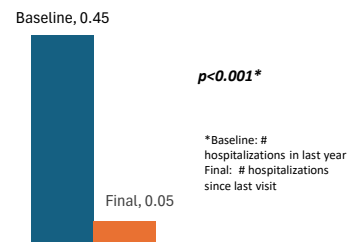


Fig 5. Average Reported Hospitalizations (N=231)



DISCUSSION

- In a 2-yr period the Concierge team completed 12,233 PAs for oncology patients including oral oncolytics and supportive care, with 81.7% PAs approved and most completed in <24 hours. Of 487 denials appealed, 72% were overturned in 3.8 days on average.
- There were 1,071 patients who opted into the PMP with average age of 68.0 (SD 14.2) and 49.7% were female.
- ADR discontinuation rate averaged 2.8% for all unique patient discontinuations compared to 8.5% for the opted-out patients, p -value <0.05.
- In a paired analysis, average patient reported hospitalizations fell from 0.45 (95% confidence interval [CI] 0.31–0.42) to 0.05 (95% CI 0.02–0.08), p -value <0.001.
- Limitations: Data were derived from several different sources. Paired data was limited to PMP patients who had both an initial assessment and reassessment. Not all patients remained in PMP over the entire study period. Hospitalization results may be due to factors other than the program.

CONCLUSIONS

Comprehensive specialty pharmacy services have a positive impact on access and clinical outcomes for oncology patients. Barriers to medication access were mitigated by PA and FA support. Hospitalizations reported per patient were significantly reduced, and there were lower ADR discontinuations in PMP patients. This program showcases a framework for other oncology programs aiming to establish an integrated specialty pharmacy PMP.

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